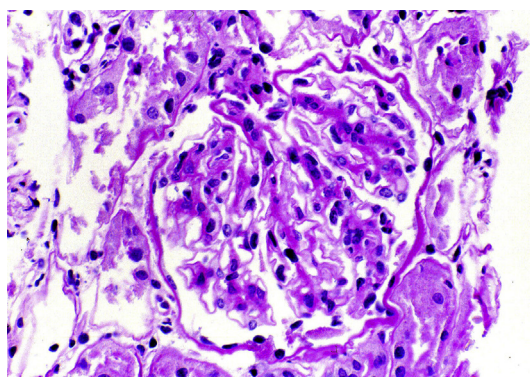


IgA Nephropathy

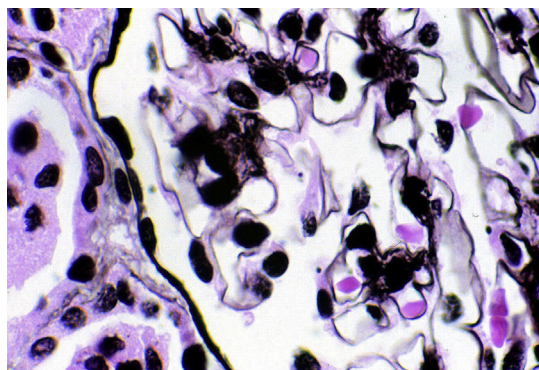
Agnes Fogo, MD

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Fig 1A. IgA nephropathy with minimal increase in mesangial matrix and cells (normal is less than or equal to 3 cells/mesangial area). The diagnosis must rely on additional immunofluorescence and electron microscopic studies ([A] periodic acid-Schiff, original magnification $\times 200$; [B] Jones' silver stain, original magnification $\times 400$).



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Fig 1B. IgA nephropathy with minimal increase in mesangial matrix and cells (normal is less than or equal to 3 cells/mesangial area). The diagnosis must rely on additional immunofluorescence and electron microscopic studies ([A] periodic acid-Schiff, original magnification $\times 200$; [B] Jones' silver stain, original magnification $\times 400$).

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Medical Photographer: Brent Weedman; with assistance from Kim Solez, MD, Joseph DeAlmeida, and Brenda Kalynchuk, of the National Kidney Foundation's cyberNephrology™ Team.

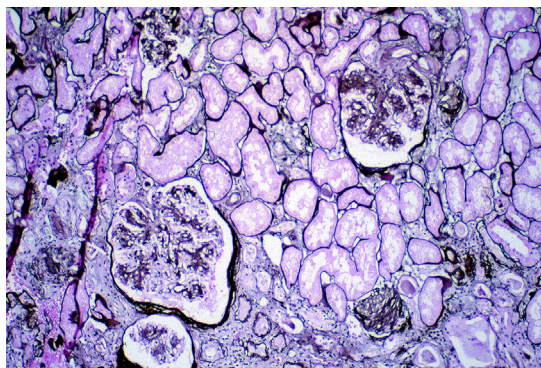
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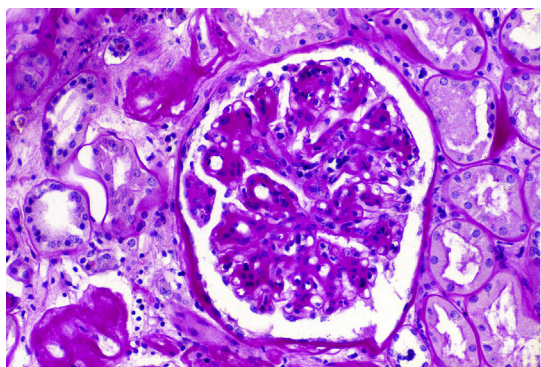
0272-6386/\$36.00

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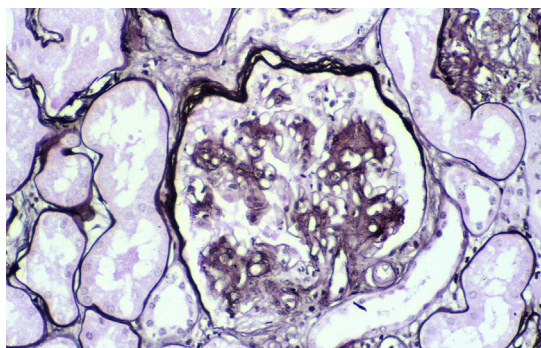
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Fig 2. Moderate diffuse mesangial expansion with increased matrix and cellularity in IgA nephropathy (Jones' silver stain, original magnification $\times 100$).



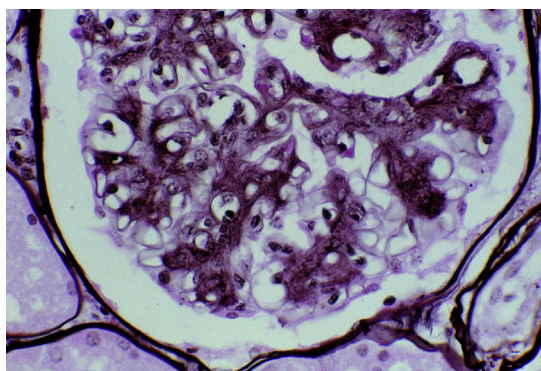
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Fig 3A. IgA nephropathy with moderate mesangial expansion with increased matrix and cells ([A] periodic acid-Schiff; [B] Jones' silver stain; original magnification for each $\times 200$).



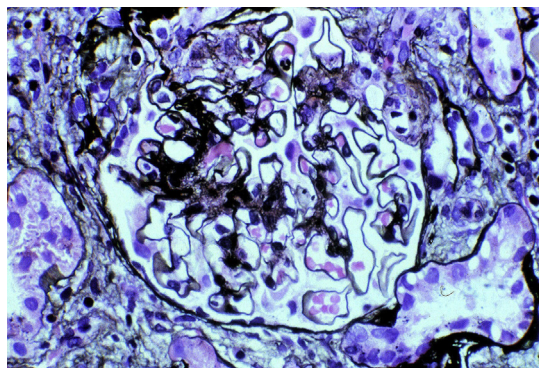
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Fig 3B. IgA nephropathy with moderate mesangial expansion with increased matrix and cells ([A] periodic acid-Schiff; [B] Jones' silver stain; original magnification for each $\times 200$).



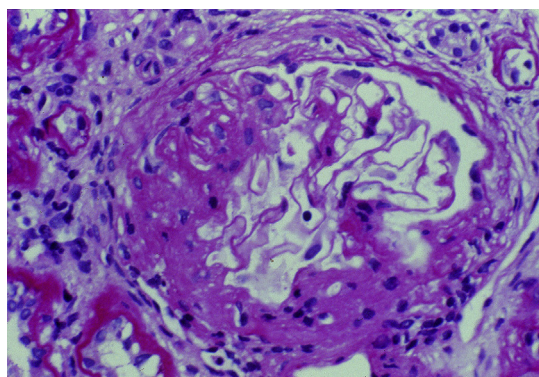
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Fig 4. Increased mesangial cellularity and matrix in IgA nephropathy (Jones' silver stain, original magnification $\times 400$).



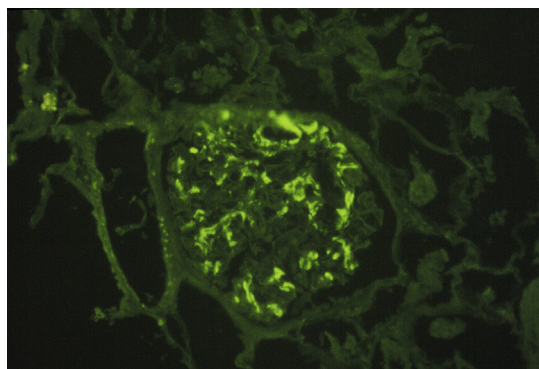
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Fig 5A. IgA nephropathy may have superimposed segmental sclerosis in addition to variable mesangial increase. (A) A small peripheral sclerotic lesion with adhesion is present; (B) more advanced segmental sclerosis is shown ([A] Jones' silver stain; [B] periodic acid-Schiff; original magnification for each $\times 200$).



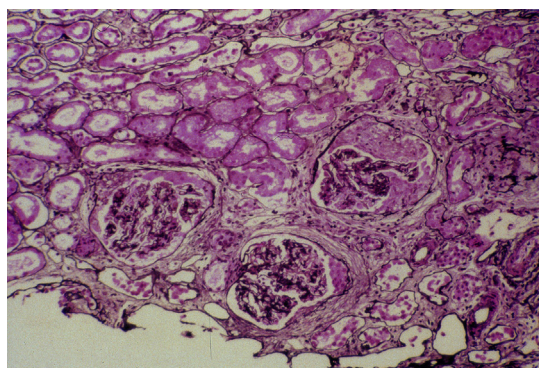
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Fig 5B. IgA nephropathy may have superimposed segmental sclerosis in addition to variable mesangial increase. (A) A small peripheral sclerotic lesion with adhesion is present; (B) more advanced segmental sclerosis is shown ([A] Jones' silver stain; [B] periodic acid-Schiff; original magnification for each $\times 200$).



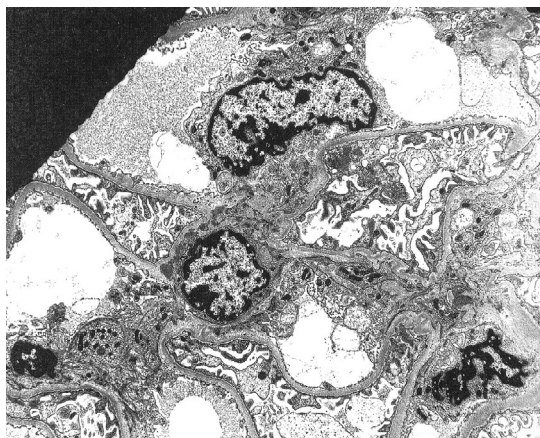
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Fig 6. The diagnosis of IgA nephropathy rests on the finding of dominant or codominant IgA mesangial deposits, as shown here. Staining may also be present for IgG and IgM, but not in greater intensity than for IgA. C3 is also often present in the mesangial areas, usually equal to or less than IgA staining (anti-IgA immunofluorescence, original magnification $\times 200$).



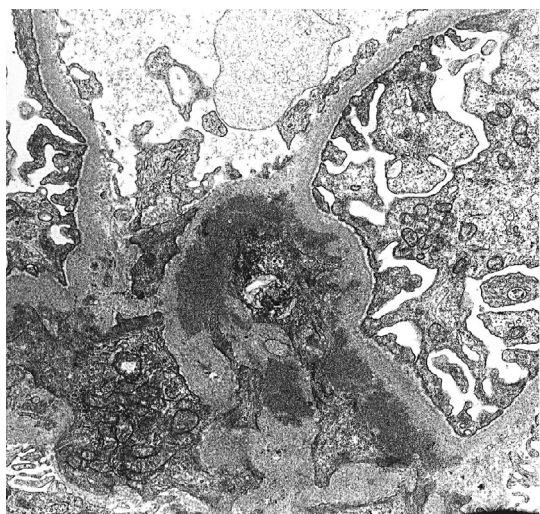
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Fig 7. Henoch-Schönlein purpura (HSP) also has mesangial IgA deposits, and thus it appears morphologically like IgA nephropathy. Clinicopathological correlation is required to distinguish HSP from IgA nephropathy, but crescents, shown here, are more frequent in biopsied HSP than in IgA nephropathy (Jones's silver stain, original magnification $\times 100$).



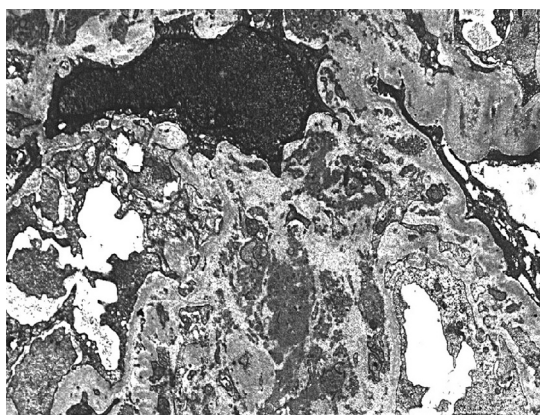
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Fig 8. Mesangial electron dense deposits and increased mesangial matrix and cellularity in IgA nephropathy (transmission electron microscopy, original magnification $\times 4,500$).



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Fig 9. Mesangial electron dense deposits and increased mesangial matrix and cellularity in IgA nephropathy (transmission electron microscopy, original magnification $\times 8,500$).



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Fig 10. Marked increase in mesangial cellularity and matrix with massive mesangial dense deposits, with some extension of deposits to paramesangial areas. (Left) There is also interposition of cells in the glomerular basement membrane with new basement membrane formation on the endothelial aspect of the interposed cells (transmission electron microscopy, original magnification $\times 4,800$).